

How Childhood and Adolescence Affect Old Age

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O joy! that in our embers Is something that doth live, That nature yet remembers What was so fugitive!

—WILLIAM WORDSWORTH

ONE ASPECT OF OUR PASTS that we tend to repeat unremembered are the experiences we absorb in childhood about other people and the world they embody. Novelist Joseph Conrad laid the stakes out somberly: “Woe to the man whose heart has not learned while young to hope, to love, to put its trust in life.”¹ If the child really is father to the man, then it’s reasonable to ask: how? This is not a simple question, obviously, but the Grant Study has been able to deconstruct it a bit, allowing us to explore the complex of childhood circumstances that shapes our sense of self and our expectations. These in turn shape the relationships that we make, and the social surround that will enrich, or not, our later years. The Study has affirmed the truth of Conrad’s warning, yet it also offers some heartening news in counterbalance, and I’ve listed this in Chapter 2 as the fifth of the great lessons of the Grant Study: What goes right in childhood predicts the future far better than what goes wrong.

Another mitigating factor, although an elusive one, are certain “sleepers” of childhood, which the Study has also illuminated. These may be deep early attachments that have been lost from view through chance, tragedy, or forgetfulness, but are brought to memory’s light again many decades later. Recovery of these lost loves can be profoundly healing, as we will see shortly in another chapter in the life story of Godfrey Camille. But there are less benign sleepers as well—genes for alcoholism, major depression, or Alzheimer’s, for example, that are present from the outset, but do not wreak their havoc until later in life. The sleeper phenomenon points up a corollary to Lesson Five: it is not any one thing for good or ill—social advantage, abusive parents, physical weakness—that determines the way children adapt to life, but the quality of their total experience.

As we’ve seen already and will see again, long-term perspectives may upset established assumptions about cause and effect, especially when these have been built on the instantaneous observations of cross-sectional research. This is what makes longitudinal studies at once so powerful and so disconcerting. In cross-sectional studies, for example, two of the most powerful correlates of successful aging are income and social class.² But wealth and status are neither the only nor the best measures by which to judge successful aging. Furthermore, our longitudinal evidence makes clear that when mid- and late-life riches do accompany successful old age, often they are not the result of financial privilege in childhood or even of other attributes popularly considered conducive to worldly success, such as looks or an extroverted personality.³ They are indirect fruits of a childhood experienced or recalled as warm and intimate, for it is such childhoods that give children their best shot at learning to put their trust in life. Many measures of success throughout life are predicted less reliably by early financial and social advantage than by a loved and loving childhood.

You’ll remember that among the College men, even achievement of military rank correlated more highly with a warm childhood than with social class, athleticism, or intelligence. For the Glueck Inner City men, whose parents were not wealthy, admired fathers, loving mothers, and warm friendships were prominent among the predictors of high income.⁴ A father on welfare, or even multiple problems in the family of origin, predicted future income and social class less robustly than a warm family.⁵ The availability of warm relationships (of which parents are ideally but not necessarily a main source) was the overriding factor.

Erik Erikson was an early student of the way children acquire Conrad’s learning of the heart. An artist/psychoanalyst, he spent many years working on the first major prospective study of child development, the Berkeley Growth Studies. Erikson believed that an infant’s first task is to develop trust and hope; a toddler’s to develop autonomy; and a five-year-old’s to become comfortable with initiative.⁶ I will elaborate Erikson’s schema of adult development in Chapter 5 ; for now, however, I will concentrate on those three early necessities and what they mean for later life.

ASSESSMENT OF CHILDHOOD ENVIRONMENT

To start with, here's some statistical context. In 1970 we set out to investigate which aspects of the College men's childhoods would predict successful mastery of hope, trust, and the resulting confidence on which autonomy and initiative depend.

In assessing childhood quality, we observed several rules to minimize bias. First, ratings were based not only upon psychiatric interviews obtained when the College men were at Harvard, but also upon interviews at that time with the men's parents, especially their mothers. ⁷ It is a limitation of the Grant Study that the men's childhoods had to be assessed retrospectively, when they were around nineteen. But it is a compensatory strength that the retrospection was very extensive—raters had access not only to the multiple-choice questions and brief essays characteristic of retrospective investigations, but to ten hours of interviews by skilled psychiatrists as well as the parent interviews.

Second, research associates involved in any aspect of the assessments were kept ignorant of the fate of the men after adolescence.

Third, we used multiple checks to ensure accurate ratings. Each man's childhood was assessed by at least two raters, and rater reliability was excellent (that is, multiple raters were highly likely to come independently to similar conclusions). ⁸ When two senior child psychiatrists reviewed and rated selected cases, their ratings too concurred with, and so further supported, the validity of the assessments of the original raters.

Fourth, we had two complete and independent assessments to cross-check against each other. The men's childhoods were first assessed in the early 1940s, and were scored on a 1–3 scale, from good to bad. Then, between 1970 and 1972, thirty years after the first assessment, I organized a second independent assessment from the same material (see Appendix C). Here's how it was done. As I've said, one constant challenge of research of this kind is the need to transform intuitions and value judgments into statistically useful data. The trick by which we had assessed marital satisfaction worked its magic on the men's childhoods: we turned a set of concrete behavioral criteria into a numerical scale. This time we rated five separate questions about the men's early years, to ensure that no single one would be weighted too heavily. These questions were:

- Was the home atmosphere warm and stable?
- Was the boy's relationship with his father warm and encouraging, conducive to autonomy, and supportive of initiative and self-esteem?
- Was the boy's relationship with his mother warm and encouraging, conducive to autonomy, and supportive of initiative and self-esteem?
- Would the rater have wished to grow up in that home environment?
- Was the boy close to at least one sibling?

Based on the material collected when the men were in college, each of these five questions was answered as superior (5), average (3), or poor (1) by two blind raters, and the two scores averaged. The 5-point ratings for the five separate questions were then added together to provide a global rating of childhood environment that could range from 5 to 25. Childhoods in the top quartile were characterized as warm; those in the bottom quartile as bleak; those in the middle two quartiles as so-so. We called the men in the top tenth the Cherished, and those in the bottom tenth, the Loveless.

One potential source of bias was that the new raters were people whose parents had raised them under Spock's permissive tutelage. They were inclined to understand the much higher strictness quotient of the College men's childhoods as a sign of parental coldness, rather than simply a reflection of the typical upper-middle-class childrearing climate of the 1920s and 1930s. I compared the new assessments with the original ones of the older raters, who had a better grasp of the mores of the time. And it turned out that I needn't have worried. The agreement between the two generations of raters was quite good.

As time went on, we were able to use this more nuanced assessment to discover, among other things, that by the time the men were into their seventh decade, the Loveless were eight times as likely as the Cherished to have experienced a major depression. We found that while even warm childhoods were often followed by heavy smoking or alcohol abuse or

regular tranquilizer use, bleak childhoods were often associated with all three. And—to return to where we started, with money—we found that the fifty-nine men with the warmest childhoods made 50 percent more money than the sixty-three men with the bleakest childhoods. These were significant findings. So was the correlation between warmth of childhood and life satisfaction. But most important of all was the fact that the Cherished were four times as likely as the Loveless—very significantly more likely—to enjoy warm social supports at seventy.

Those are abstractions, however. Here are pictures of what the childhoods of the Cherished and the Loveless really looked like, and of the effects they exerted on the men's futures.

THE LIFE OF OLIVER HOLMES

Judge Oliver Holmes had one of the warmest childhoods in the Study. He received a score of 23 out of a possible 25 on our Childhood Rating Scale. The important people in his life had made his childhood a pleasure, and their affectionate and empowering warmth lived on in his own parenting and even his children's. I have told much of Holmes's story in *Aging Well*, but I will reprise selected portions here because it is such a vivid illustration of the power of love.

Holmes's childhood placed him firmly amongst the Cherished. His parents were well-off, and they spent their money on music lessons and private schooling for their children, not on luxuries for themselves. At fifty, Holmes wrote, "My parents gave me wonderful opportunities." His relatives were nurses, music teachers, YMCA directors. No one in his family was known to be mentally ill. The Holmeses were Quakers, but Mrs. Holmes never stood over her children and made them say prayers; she felt, as she explained to Lewis Gregory in 1940, that "religion must be taught by example rather than by words." Besides, she went on, "Oliver was always cooperative and reasonable and almost never had to be punished. He has a delightful sense of humor."

Gregory saw Judge Holmes's mother as "a rather serious person with a great deal of kindness and gentleness," and she noted an example.

During the interview Holmes's younger brothers were in and out of the living room, playing football on top of us, aiming BB guns at our heads, but it didn't take Oliver's mother long to have the situation well in hand. I would call her a wise person, an intelligent mother, and calm in emotional makeup.

Oliver got into frequent fights with other boys when he was in grammar school. One time he was knocked unconscious with a rock, but—ever the optimist—he reassured his parents that the other fellow had gotten the worst of it! In college, even while he was a member of the Pacifist Society, he still enjoyed aggressive games, and belonged to the judo club and to a debating team. It is easier to grow up to be a Quaker if when you are young your parents can accept your assertiveness—and have a sense of humor. It's easier to grow up to be a confident and competent individual, too.

Holmes's father had put himself through medical school. He became a top orthopedic surgeon, and Mrs. Holmes said about him that "his fellow physicians don't see how he can be so genuinely concerned about his patients. . . . He has so much sympathy for people who are ill and in trouble." Holmes described his father as "generous and never encroaching on a person's individuality."

The Holmeses were a close-knit family, and Holmes considered his parents among his best friends. After Oliver completed law school, his father bought him a house in Cambridge, just fifteen blocks away from his own. Oliver's wife's parents lived even nearer; for just as greatly as Holmes admired his father did Cecily admire hers. At age sixty-five, Judge Holmes described his brother's family as "so nice that one would have to be a stone not to enjoy it." Yes, his brother lived in Cambridge too.

I can hear my readers muttering, "I knew it. This guy is a patrician judge. His father bought him an expensive house. No wonder he's a picture of contentment in his old age! What did he ever have to worry about?" This reaction is exactly why social science needs statistics as well as evocative case histories. We found that contentment in the late seventies was not even suggestively associated with parental social class or even the man's own income. What it was significantly associated with was warmth of childhood environment, and it was very significantly associated with a man's closeness to his father. Holmes was very fortunate, there's no doubt about that. But the best part of his good fortune was not financial.

When I re-interviewed the Holmeses at seventy-eight, Judge Holmes was still working several days a week in an effort to reform the Massachusetts judicial system. He and his wife had recently sold their Cambridge house and were

living in a retirement complex. Their living room was warm and welcoming, with an upright piano and a working fireplace, and the walls were covered with Cecily Holmes's very good watercolors and photographs of the children and grandchildren.

Judge Holmes in old age was a tall, slightly balding man. At first he reminded me of a distinguished but very uptight professor of neurology I had once known, but as he talked the depth and complexity of his humanity came through. Holmes always focused on the positive side of people—not as someone who sees the world through rose-colored glasses, but in the way that a clear-eyed but loving parent appreciates his child. I was struck by how comfortable I felt with the Holmeses. Our interview lasted three hours, but they betrayed no impatience. They laughed frequently, and it wasn't nervous or social laughter, but the kind that comes straight from the belly.

This was a responsible and venerated judge, still working at seventy-eight. He knew how to stand up for himself. But he knew how to play, too, and he knew how to depend on others, as his interactions with his wife made very clear. Unlike many of the Study men who could barely list one intimate friend, Holmes listed six with whom he shared "joys and sorrows."

Seven years later, at eighty-five, Judge Holmes still experienced his health as "good" and his energy as "very good," although he could no longer walk for two miles or climb stairs without resting. He had to get up three times to urinate during the interview, but his description of his medical problems displayed what his college interviewers had recognized as "whimsical humor" sixty-five years before. It was his prostate, Holmes explained dryly. "My doctor admires its size." When Holmes was eighty-nine, and I was summarizing for this book the hundreds upon hundreds of marriage ratings we had collected over seven decades (see Chapter 6), his marriage scored among the four best in the Study. For the last ten years Judge Holmes in counterpoint to Cecily's painting has been writing beautiful love poetry to his wife.

Holmes's is as good an example as we have in the Study of a warm childhood. How do we know that it predicts success, though? And how can we know how much it was the security of warmth and how much the security of money that allowed Holmes to become what he did? Furthermore, what contribution do genes and hormones (oxytocin, for example) make to mental health and the capacity for empathy and love? We don't know exactly (yet) what the interplay among these factors is. Certainly good fortune in any aspect of the "nature" lottery may exaggerate the effects of a warm childhood on later outcome, and I will use Study statistics to untangle some of these threads as we go on. In the meantime, suffice it to say that the powerful effects of a caring family can sometimes be discerned best in their absence, as the following life story shows.

THE LIFE OF SAM LOVELACE

When he first entered the Grant Study in 1940, Samuel Lovelace was scared. "Sam's anxiety is far in excess of the average Grant Study man," the examining physician wrote. Even at rest, his pulse was 107. The Study staff described him as an immature lad who "tires rather easily," and they were disturbed by his diffidence and his "inability to make friends."

Six years into the Study, a staff member summed Lovelace up as "one of the few men to whom I would assign the adjective selfish It was as if he were looking out of a very small gun barrel." That was a warning; "selfishness" in the Grant Study men appears to have been the result not of too much love in childhood, but of too little. More on this shortly.

No one looking at Sam Lovelace's whole life could possibly regard him as selfish. Even when he was in college, there were some Grant Study staff who were touched by him. They called him a "nice boy . . . intelligent, warm, open chap . . . with potential ability that has not yet found an avenue through which he could direct it." But his childhood environment score was 5 out of 25, a score that put him in the bottom 2 percent of the Study for childhood warmth. Even Godfrey Camille's childhood got a 6.

Sam Lovelace entered life as the result of an accidental pregnancy in a family that was solidly middle-class, even if it didn't possess quite the wealth of the Holmeses. But remember that while warm childhood correlated very significantly with a high Decathlon score, social class was statistically unrelated. And class was the least of Sam's problems. By his mother's own testimony, he never got much attention, even of the most fundamental and presumably nonnegotiable kinds. Asked by Lewis Gregory what she would do over again in bringing him up, she replied, "I would try to provide better care and feeding during infancy . . . and more companionship." She also said of her two sons that she had "always

expected them to be adults.” When asked at thirty what he wanted his children to have that his childhood had lacked, Lovelace replied, “A richer environment in terms of stimuli.”

Sam grew up feeling that he “didn’t know either parent very well,” and recollected “little demonstration of affection from either of them.” They, in turn, dismissed him as “too dependent.” But the most independent and most stoical men in the Grant Study were the men who came from the most loving homes; they had learned that they could put their trust in life, which gave them courage to go out and face it. I use the term stoical to indicate people who use the adaptive involuntary defense of suppression, which I’ll discuss in Chapter 8. As a child, Sam spent most of his time with his dog, and he “grew apart” from his only brother. His mother believed that “people like Sam more than he likes them.”

Sam got all A’s in high school and was editor of the school newspaper. To the Grant Study staff he appeared “graceful and coordinated.” But his parents focused on their belief that he was “not good at sports, and he hates those that he does.” For many years Sam Lovelace was a stranger to games; only late in life did he learn how to play.

In the early interviews of his college years, Sam described his mother as “very moody, unpredictable, and given to worry. . . . I don’t feel too close to her.” He had little respect for his father. At forty-seven, he remembered his mother as “pretty tense and high-strung,” and his father as a “remote, anxious, tired” man and a bigot. He visited his parents often as an adult, but not so much out of love as out of his fear that they would soon die. Often they ended up in arguments over politics, which reduced his conservative parents to tears and left the more liberal Sam feeling once more on the other side of an unbridgeable gulf. He took little vacation time—“play” still wasn’t easy for him—and spent most of it in dutiful visits to relatives.

When I met Samuel Lovelace at fifty, he was a distinguished-looking man, neatly dressed in suit and bow tie. His hair was gray; as in college, he looked older than his years. During the interview he smoked incessantly and gazed out the window, making no eye contact. Since I never got a smile from him or even a direct gaze, it was hard to feel connected with him. The reasons were not hard to understand; he didn’t feel connected either.

He was finding love as hard to come by in adult life as he had in childhood. At nineteen, he had said of himself, “I don’t find it very easy to make friends,” and at thirty that it was “difficult to meet new people.” At fifty, nothing had changed. He described himself as “sort of shy,” and told me that he didn’t socialize much. At work (he was an architect) he felt bullied and manipulated by his boss. I asked him who his oldest friend was, but he told me instead about a man whom he greatly envied. In the twelve months preceding our interview, he and his wife had invited no one over to their house.

Lovelace’s lack of hope and trust in other people made him extremely vulnerable to loneliness, which was one of the enduring issues of his adult life. His marriage to a chronically alcoholic woman was desperately unhappy. His mother had nagged him into going to church when he was young, but he had quit as soon as he escaped to college—another opportunity for community lost. His discomfort in social situations kept him from joining organizations, and so did his apparent dislike of games (which, as his later life demonstrated, probably reflected social discomfort far more than the lack of skill his parents so insistently attributed to him).

Lovelace approved of hippies on the grounds that “I’m for anything that will shake up the adult world”; the only positive feeling that he harbored about the status quo was the hope that it would change. At eighteen, such a philosophy is healthy, but at fifty, as I noted in a paper on the twenty-fifth reunion poll of the class of 1944, it is correlated with social isolation and with seeking psychotherapy.⁹ Indeed, other people were so lacking in his life that Lovelace’s chief source of comfort at that time was the psychiatrist whom he had visited for fifteen years.

The unloved often have a special capacity to identify and empathize with the pain and suffering in the world, and so it was with the self-doubting, pessimistic, hippie-loving Lovelace, who had opposed Joe McCarthy, supported Adlai Stevenson, and believed American military involvement was no solution to the problem of Vietnam. While many conservative men with happy childhoods wished to slow down the process of racial integration in the country, Lovelace wished to hasten it. Still, an essential passivity and sense of isolation kept him from acting. “Although I take a liberal stance at cocktail parties, I can’t follow it out onto the streets,” he said. Sometimes he didn’t even vote.

In Freud’s psychoanalytic worldview, orality characterizes the earliest stage of psychosexual development. It is the period during which, allegedly, the mouth is the infant’s primary erogenous zone and the focus of all libidinal interests

and conflicts. But orality is more usefully seen as a metaphor for the longing of hearts that have not learned to fill themselves with hope and love, as Erikson recognized when he wisely reframed its developmental correlate as lack of basic trust. 10 It was perfectly true that Lovelace was a man of many oral habits. He used stimulants to start the day, inhaled several packs of cigarettes to see him through it, and drank eight ounces of bourbon to soothe its end. Then he took three sleeping pills to go to sleep. It was also true that as a child he had bitten his fingernails, and he still occasionally put his thumb into his mouth as he talked with me. But none of this was the point. The main legacy that Sam Lovelace's parents had left him was not literal hunger, but a profound lack of trust in them or anyone else, including himself. What was spoiling Lovelace's life was fear.

At thirty-nine he wrote, "I feel lonely, rootless, and sort of disoriented." He said of his painful marriage: "No matter how hollow a marriage it may be, it still gives one a home and a place in society. . . . Despite even some hatred, it's easier to suffer with Janie than to suffer without her." He confessed that he was "afraid of giving up marriage and standing on my own," and one reason for his fear was that his wife was much richer than he was. Clearly he wanted something better, but he was afraid to try to find it. Initiative and autonomy are very difficult when the world is such a perilous place, but Lovelace berated himself for his inadequacies.

I interviewed Lovelace again when he was fifty-three, after the death of his alcoholic wife. I wanted permission to publish his story, and I thought it was important to make that request in person. With the burden of his agonizing marriage relieved, his life was one important degree less dangerous, and he was no longer the cadaverous, worried man I had met three years before. He—almost—had a glow in his face. He was now president of the Louisville Home for the Aged, and had sublimated his fears of aging and death (as marked in him as in any man in the Study) in pride at giving the older residents of his city a better quality of life. His social life had improved, if only slightly. He gave me permission to publish his bleak history, which was certainly an altruistic act.

Even having seen the changes in Lovelace over his first three years as a widower, however, I was not prepared for the man I interviewed just before his eightieth birthday. He was much less depressed. He looked younger than he had twenty-five years before, and when I said something about that he told me that I was not alone in thinking so. Except for the antidepressant Zoloft, he had given up the tranquilizers, sleeping pills, and stimulants of his youth. He had had girlfriends after his wife died. Like many of the Loveless men, it was hard for him to let love in, and for many years he had eluded any serious attachment. But at sixty-three he finally succumbed and remarried. Compared to the other men's, his second marriage scored in the "unhappy" category. Nevertheless, at seventy-nine he told me that his wife was the best thing that ever happened to him. As we'll see in Chapters 5 and 7, brain maturation leads to an increased capacity for intimacy, especially when that has been retarded by emotional privation. Even this ambiguous marriage was an improvement over anything he had ever had before, and he could appreciate and rejoice in that.

In his chronic withdrawal and lack of energy, Lovelace had had little room for personal interests. But now, warmed perhaps by his new marriage, the man who had been a "failure" at sports throughout his life was (his wife assured me) a fabulous dancer. Lives change, and things can get better. But the people who don't learn to love early pay a high price in what Conrad calls woe. And, as I'll show in a brief statistical foray, Lovelace remained to the end one of the most loveless men in the Study.

IMPORTANT LINKS BETWEEN CHILDHOOD AND OLD AGE

The men at fifty. When the College cohort was about fifty, I explored in a series of analyses the relationship between childhood environment and adult outcome. The results confirmed the qualitative message of Holmes's and Lovelace's stories—that for good or ill, the effects of childhood last a long time. Half the men from the best childhoods, but only an eighth from the worst, had made what we then considered optimum adult adjustments. Only a seventh of the Cherished, but fully half of the Loveless, had at one time been diagnosed as mentally ill, under which rubric we included serious depression, abuse of drugs or alcohol, and a need for extended psychiatric care (more than 100 visits) or hospitalization. The Loveless were five times as likely as the Cherished to be unusually anxious. They took more prescription drugs of all kinds, and they were twice as likely to seek medical attention for minor physical complaints. They spent five times as much time in psychiatric offices.

Some readers may object to my classifying a hundred visits to a psychotherapist as a statistical manifestation of mental illness. There are lots of reasons that healthy people go to psychiatrists, you might point out. What about young

psychotherapists who seek analysis as part of their training or professional development, for instance? Here I'll avoid a he-said she-said argument, and let the Grant Study's telescope provide a statistical resolution. Of the twenty-three men with the lowest Decathlon score, nine had seen a psychiatrist one hundred times and only seven never. Of the thirty men with Decathlon scores over 6, only three had ever seen a psychiatrist at all and none more than ninety-nine times—a very significant difference. Of the five psychiatrists in the Study, all had seen a psychiatrist over one hundred times. Even if for the sake of argument we accept that as irrelevant, four had also used harmful amounts of psychoactive drugs and three had experienced a major depression. This is not to say that having once been considered less than mentally robust by the Study a man was doomed to carry that label forever; Godfrey Camille did not. Nor does it mean that people can't turn their lives around; Godfrey Camille did. But it does indicate that people who seek a great deal of psychiatric care have something in their lives that needs turning around. People whose lives feel just fine tend to eschew psychiatrists.

In individual case records, we frequently observed the following unhappy sequence. A poor childhood led (first) to an impaired capacity for intimacy, and (second) to an above-average use of mood-altering drugs. Warm childhoods clearly gave the more fortunate men a sense of comfort with and acceptance of their emotional lives that lasted into old age, while bleak early years left their survivors with (third) an enduring apprehension about trusting and facing the world by themselves. The fourth and cruelest aspect of bleak childhood was its correlation with friendlessness at the end of life. The Cherished were likely to be rich in friendships and other social supports at seventy—five times more likely than the Loveless who trusted neither the universe nor their emotions, and remained essentially friendless for much of their lives. The Loveless Adam Newman, for example, who insisted that he never knew what the word “friend” meant, had managed to find a wife who fully satisfied his limited emotional desires. But she was his only friend, and he was all too uncomfortably aware that her death would leave him essentially alone.

The Cherished were eight times more likely to report good relationships with their siblings, and more likely to achieve the wider social radius that Erikson called mature or generative. I will admit that I feel some uncertainty about whether childhood environment is solely responsible for this; I suspect that there are genes for loving natures, undiscovered though they yet be. Here again, time will tell.

Let me turn now to the corollary of this chapter's lesson—that it is the quality of their total experience that determine the way children adapt to life, not any single piece of good or ill fortune. The Grant Study turned up no single childhood factor that predicted well-being (or its opposite) at fifty. It was the number, or perhaps the constellation, of positive and negative childhood factors that predicted mental health risks, not any particular ones or any pattern among them. But the total mattered.

At about age fifty, the men were asked to respond to 182 true-or-false statements in the Lazare Personality Scale. 11 Eight of those statements very significantly distinguished the men who thirty years later would receive the lowest Decathlon scores from the men who received the highest. That is, they distinguished the lonely, the unhappy, and the physically disabled from the happy, successful, and physically well. Remarkably, the same eight midlife questions also very significantly distinguished the men who had been classified as Loveless and Cherished thirty years prior to testing.

- My behavior with the opposite sex has led to situations that make me anxious.
- I have often thought that sexually, people are animals.
- I usually feel that my needs come first.
- Others have felt that I have been afraid of sex.
- I easily become wrapped up in my own interests and forget the existence of others.
- I put up a wall or shell around me when the situation requires it.
- I keep people at a distance more than I really want to.
- I have sometimes thought that the depth of my feelings might become destructive.

What does this have to do with childhood? These eight revealing questions address a fundamental discomfort with the emotional aspects of life, and a resulting self-doubt, pessimism, and fearfulness. Men with warm childhoods subscribed to very few of these statements, but (very significantly) the less fortunate men often subscribed to four or more. And the

more at ease the men were with their feelings, the more successful they were at the rest of their lives.

The College men who did not achieve successful or gratifying careers revealed a life-long inability to deal with anger, which I have documented in *Adaptation to Life* and also in a statistical examination of the work lives of the Inner City men. 12 Anger is a delicate balancing act. No one can avoid anger forever and hope to prosper in the real world, but discomfort with aggression is a developmental challenge not only for women and overeducated men; it was for the Inner City men, too.

So how does a child learn to trust what he feels and other people's responses to his feelings? When you're just getting the hang of grief, rage, and joy, it makes all the difference in the world to have parents who can tolerate and "hold" your feelings rather than treating them as misbehavior. 13 Maybe Mrs. Holmes's willingness to let her two sons act up in the living room was a coincidence, but I doubt it.

And what if you don't have parents like that? How do you learn such things if you can't engage comfortably with other people? How do you tackle the world confidently, or risk finding someone to love, or even find room among your fears to relax and pay attention to things other than yourself? There was no man in the Study who had a bleaker childhood than Sam Lovelace. All the circumstances in his constellation conduced to a heart that didn't know how to be satisfied. His Childhood Environment score was a rock-bottom 5, and he alone answered "True of me" to all eight of the questions. His Decathlon score was 0, and at age seventy there were only two men in the Study with fewer social supports than he. It's not enough to be loved; you have to be able to let love in. As Sam's mother had told us when he was eighteen, "People like Sam more than he likes them." But liking—trusting is probably more the issue—has to be learned, and Sam didn't learn it at home.

HEREDITY VERSUS ENVIRONMENT

In 1977 I wrote, "If isolated trauma did not affect adult life, chronically distorted childhoods did affect adult outcome. Although the mental health of relatives was not related to subsequent psychopathology in the men, the mental health of their parents was. The Worst Outcomes were twice as likely as the Best Outcomes to have a mentally ill parent. This effect would seem to be mediated environmentally." 14

In retrospect, though, I can see that environmentalism in the postwar social sciences was just as extreme as the pre-war hereditarianism had been. This is a good example of the way scientists change science and then are changed by science in their turn. Note too that both the turn to environmentalism and the recent reaction away from it in favor of the neurosciences are encompassed in the lifespan of the Grant Study. Longitudinal studies are inescapably subject to such developments; this is one of their infuriating complications and one of their invaluable advantages. So as the years passed, I had to reconsider some of my nature vs. nurture convictions. Once again, I turned to prospective study to provide accurate data, and to reveal and redress the cultural biases of previous investigators (including myself).

For instance, data that I gathered in 2001, when the men were turning eighty, indicated that the mental health of relatives (not only parents) was an important fork in the road leading from a warm childhood to adult mental health. The presence of mental illness in the family was a wild card with the potential to trump even the most auspicious nurturance. That pointed to hereditary, rather than environmental, influence.

Table 4.1 summarizes some of that analysis. It shows mental health plotted against three conditions: bleak childhood environment, familial mental illness, and personality rating at age twenty-one. I've described already how we assessed childhood environment; the personality assessment at age twenty-one was part of the original investigation of the men during their college years. The scores for the second condition, familial mental illness, were established this way. In a process that will be familiar by now, we reduced four concrete indicators of genetic vulnerability—alcoholism, depression, poor familial longevity, and, surprisingly, early death of the maternal grandfather—to 3- or 4-point scales. (It can be surprisingly difficult to assess familial longevity accurately, but we were able to do it. The men's parents knew the dates of death of their parents, and the Study lasted the necessary six decades until the last parent of a Study man died in 2001. Another gift of lifetime studies. Familial longevity was estimated by summing the ages of the oldest parent or grandparent on the maternal and paternal sides.)

Because these four disparate variables correlated significantly with each other, they were summed to yield a heredity score of 0 to 12, which, as Table 4.1 illustrates, did have some predictive power. These scores did not reflect any kind of

genetic testing or even precise diagnoses. They were pragmatic and crude estimates of the presence of three broad syndromes in a family—alcoholism, depression, and short life. Longevity of the maternal grandfather was included individually because its absence had a very significant—and also a fascinating and difficult to explain—association with both neuroticism and depression; much more on this in Chapter 10 . It’s worth noting that a high heredity score was very significantly correlated with a bleak childhood, and my work on alcoholism later demonstrated that heredity accounts for almost 100 percent of familial transmission of alcoholism (see Chapter 9). Holmes was blessed with a heredity score of 0, while both Camille and Lovelace had significant genetic loading for mental illness.

Table 4.1 Important Associations of Heredity and Childhood Environment with Adjustment to Life

Very Significant = $p < .001$; Significant = $p < .01$; NS = Not Significant.

* This was a sum of five variables leading to vascular and heart disease (high diastolic blood pressure, Type II diabetes, obesity, heavy smoking, and alcohol abuse). The vascular risk variables are discussed in Chapter 7 .

The first two columns of the table illustrate the correlations of childhood environment and heredity with various maturational and situational circumstances in later years. A warm childhood predicted later social and love relationships better than hereditary factors did. But heredity was the better predictor of later health-related developments like alcohol abuse, smoking, and, most dangerously, the vascular risk variables that I’ll discuss in Chapter 7 . Why heredity was less associated with adjustment in old age is puzzling. One factor may have been selective attrition by early death among the alcoholics and depressives.

The third column of the table reflects another curious chapter in the history of Woods’s equivocal personality trait schema—an arresting tale of a scientific ghost and the inestimable value of long perspective. In the mid-eighties, Paul Costa and Robert McCrae, longtime senior investigators at the National Institutes of Health and the Baltimore Longitudinal Study of Aging, established an inventory of personality traits that became very well known under several names: the NEO, the Big Five, and the Five Factor model. 15 That first name is an acronym for the first three of its five traits—Neuroticism, Extraversion, Openness, Agreeableness, and Conscientiousness; the second and third are self-explanatory. Among the statistically inclined, the Big Five has emerged as a robust model for understanding personality, but clinicians, including me, have found it less useful. One of my reasons is that McCrae and Costa have used it to argue that personality does not change over time. 16 This is a point of view clearly not in accord with my Grant Study experience, and one that has been sharply contested in other quarters as well. 17

In 1998 Stephen Soldz, a psychologist expert in statistics and in the Big Five, thought to look back to Woods’s earlier twenty-six-trait classification and the way it was applied to the College men in the 1940s. As I’ve said, Woods’s methodology was not all it might have been, and the usefulness of his schema has been slight, with the one startling exception that I described in the last chapter. Nonetheless, out of the material from that time, Soldz was able to extrapolate five traits that correlated, with high agreement among seven independent raters, with the Big Five. In his hands, these five traits were highly predictive of most of the outcomes in Table 4.1 , a result that forced me to reconsider my distrust of the Big Five’s predictive power. There was also a high correlation between Soldz’s traits and the College men’s results on the Big Five proper, which was administered to the College men at the age of sixty-seven. 18

It was fascinating to watch a measure from the very earliest days of the Study—one that for a long time looked almost laughably useless and that redeemed itself only by the skin of its teeth with the surprising success of Well Integrated—receive a whole new lease on life in the hands of a truly sophisticated statistician. Woods tried something. It mostly didn’t work, and the material that he amassed wasn’t much use in his own context. But if he hadn’t tried, his material would not have been available once a larger context appeared. His reach exceeded his grasp, but that overreach is an important attribute of long-sighted and imaginative science.

In another “However,” though, recent twin studies at multiple universities, especially those by David Lykken at the University of Minnesota, have shown that there is a major genetic component to Big Five test scores—perhaps up to 50 percent. 19 This implies that many personality characteristics that people tend to attribute to family and societal influences (including ones as unlikely as spirituality) are at least in part genetic. It also means that the “Bleak Childhood Environment” in Table 4.1 reflects contributions from hereditary factors as well as environmental ones. This is a major complication for the analysis of our data, and another issue that we will have to wait for time to sort out.

The third column in Table 4.1 refers to the NEO-related scores. It is more speculative than the other two, but it suggests that a high score on the Big Five trait of Extraversion (that is, thriving on challenging environments, social interactions, and keeping busy) and a low score on the trait of Neuroticism (that is, anxiety, hostility, depression, and self-consciousness) predicts a high Decathlon score. Indeed, the association (for the statisticians, $\rho = .45$) was as strong as the benchmark correlation between people's height and their weight.

Some of the traits that the men had been rated for in college—Humanistic, Vital Affect, Friendly—had been significantly associated at that time with a soundness rating of A. Others—Inhibited, Ideational, Self-conscious and Introspective, Lack of purpose and values—had been significantly associated at the time with a college adjustment rating of C, or unsound. 20 In midlife, however, none of these adolescent traits were associated positively or negatively either with psychopathology or with good adjustment. In retrospect, the traits associated with poor college adjustment seem to have been part of normal adolescence (which adults do sometimes view as pathological!). Surprisingly, the one trait that proved to be strongly associated with healthy midlife adjustment was a trait rather uncharacteristic of adolescents, called Practical, Organized. It included the ability to delay gratification, and proved to be strongly associated with healthy midlife adjustment, but not after age seventy-five. On the other hand, we found a “sleeper” trait—one that didn't appear to be important in my first foray when the men were forty-seven, but became notably so afterward. This was our old friend Well Integrated, which I defined in Chapter 2, and which, while not so important in midlife, significantly predicted a physically active and cognitively intact old age. The reason for this appears to be that Well Integrated was the one Woods variable that (like a warm childhood) independently predicted the absence of vascular risk factors like smoking, obesity, and elevated blood pressure. Self-starting was another trait not important early in midlife but very important at the end of life.

What does it mean that a variable is predictive at one time of life but not at another? It could mean that what once looked significant really wasn't, or vice versa. But it could also mean that some things really are significant at certain times, but not at others. This makes prediction a much more complicated issue, and it also points up a crucial aspect of lifetime studies. They never let us forget that something that is true at one time of life may not necessarily be true at another. The longer our view, the better a chance we have to figure out why some correlations endure and others don't. Sometimes the missing link is in our data; sometimes it's in our science; sometimes it's in our intuition. But even during the (admittedly uncomfortable) periods when we know a shift has taken place, or a predictor has stopped predicting, but we don't know why, at least we know that it's happened. It is only by studying lives over time that we become aware of changes like this, which may be markers of important developments, such as physical maturation in the brain. The vicissitudes of associations and correlations that we can follow only in longitudinal studies remind us to keep our eyes and our minds open and to guard against premature closure.

MOTHERING VERSUS FATHERING

Here's an example of how these vicissitudes look. When I first began to analyze the effects of childhood upon adulthood, it looked like the total childhood environment was more important than the maternal relationship per se; as a psychiatrist, I found this surprising. After eighty, however, the men's childhood relationships with their mothers look more significant—another “sleeper” effect. What its meaning might be we don't yet know.

The Study found some facets of adulthood in which a good relationship with one parent or another exerted the more important influence. As the men approached old age, their boyhood relationships with their mothers were associated with their effectiveness at work, but their relationships with their fathers were not. A man's maximum late-life income was significantly associated with a warm relationship with his mother, as was his continuing to work until seventy. His military rank and his inclusion in Who's Who were also marginally significantly associated with a warm relationship with his mother.

A warm childhood relationship with his mother was significantly associated with a man's IQ in college, and, more important, with his mental competence at eighty. A poor relationship with his mother was very significantly, and very surprisingly, associated with dementia. For example, of the 115 men without a warm maternal relationship who survived until eighty, 39 (33 percent) were suffering from dementia by age ninety. Of the surviving men with a warm maternal relationship, only 5 (13 percent) have become demented—a significant difference. In the Grant Study, dementia has not been significantly associated with vascular risk factors. One senior colleague of mine insists that this finding must be

wrong, on the grounds that no one has noted it before. He forgets that seventy-year prospective studies are as rare as hen's teeth. Only time—or replication—will resolve the matter.

None of these issues were even suggestively associated with the quality of the man's relationship with his father. However, warm relationships with fathers (but not with mothers) seemed to enhance the men's capacity to play. Men with warm paternal relationships enjoyed their vacations significantly more than the others, employed humor more as a coping mechanism, and achieved a very significantly better adjustment to, and contentment with, life after retirement. Counter-intuitively, it was not the men with poor mothering but the ones with poor fathering who were significantly more likely to have poor marriages over their lifetimes. All five of the men who reported that marriage without sex would suit them had poor paternal relationships, but these men were evenly distributed as to the adequacy or inadequacy of their mothering.

Men with good father relationships also manifested less anxiety—a significantly lower standing pulse rate in college, for example, and fewer physical and mental symptoms under stress in young adulthood. Men with poor father relationships were much more likely to call themselves pessimists and to report having trouble letting others get close. And good father relationships very significantly predicted subjective life satisfaction at seventy-five, a variable not even suggestively associated with the maternal relationship.

Nonetheless, a mother who could enjoy her son's initiative and autonomy was a tremendous boon to his future. Judge Holmes was by no means the only successful man whose mother admired his assertiveness. Other mothers of successful sons boasted: "John is fearless to the point of being reckless"; "William could fight any kid on the block. . . . he was perfectly fearless"; and "Bob is a tyrant in a way that I adore." Not all the men were so lucky; certainly Frances DeMille (in Chapter 8) was not. Yet as the men moved into adulthood, their mothers, as if by magic, were remembered as progressively weaker, and their fathers as progressively more dominant, childhood figures.

RECOVERY OF LOST LOVES

No one whom we have loved is ever totally lost. That is the blessing and the curse of memory. As Tolstoy wrote, "Only people who are capable of loving strongly can also suffer great sorrow; but this same necessity of loving serves to counteract their grief and heals them." 21 There's a distinction to be made here between privation and deprivation. Privation—never having loved or been loved—leads to psychopathology, not to grief. Deprivation—the loss of those whom we have loved or been loved by—leads to tears, but not to sickness. Grief hurts, but it doesn't kill.

The psychodynamic work of mourning has less to do with saying good-bye and letting go than with bringing down from the attic the old pictures that re-evoked forgotten moments of intimacy. I believe, though it has not yet been proven, that the primate brain is constructed to retain love, not to relinquish it. It is much easier to conceptualize the searing pain of loss than the subtle and almost imperceptible ways that we absorb the people we love into ourselves, and perhaps this is one reason that some psychotherapists emphasize the pain of grief over its work of remembrance. But dynamic psychotherapy must attend as carefully to the recovery of lost loves as it does to the experience of pain, or, worse, the retrieval of old resentments.

The recent years of the Grant Study have shown that our lives when we are old are the sum of all of our loves. It is important, therefore, that we not let any of these go to waste. One task of the last half of life is to recover the memories of the loves of the first half. This is an important way that the past affects the present. The rediscovery of a love once lost, or of the power to forgive, may be a source of great healing. To participate in the recovery of others' lost loves is yet another joy of conducting a longitudinal study.

I will revert once more to pictures to convey this joy. Godfrey Camille, whom we met in Chapter 2 , wrote when he was eighteen, "I am sorry that my family was so terribly strict with me from the ages of 5 to 12." He was charitable about this, but realistic: "They can't be blamed for it because they were brought up in 1890 New York. . . . Dad and I have never gotten on very well."

Over the years, as Camille uncovered his past through psychotherapy, he tried to help me understand what that uncovering meant to his life.

Is it the re-finding of the love that matters? Or is it the chance to re-celebrate and to re-celebrate again and again the bond that holds? An image comes to mind—the empty wine bottle used as a candlestick. The wine may have been the initial warmth in life, but once it is gone only the cold and empty glass bottle remains—until in memory we re-light the candle of conviviality, and as it drips, it transforms the symbol of the warmth that was spent into a differently shaped, differently colored object—alive with new warmth. Early loves may be “lost” because they were taken for granted and never reinforced by review. Recollection and retelling have a way of making them increasingly visual and real. It is the visual perception that’s particularly effective when it comes to learning.

Long before he wrote those words, however, Godfrey Camille had engaged himself in the process of turning the cold, empty bottles of his life into vessels of new light, hope, and strength. At sixty-five he reminded me (as if I could forget) that “empathy was not my father’s strongest suit,” and he offered an old, but newly remembered, example. Having climbed a cherry tree to look at the blossoms, he lost his balance and fell twelve feet to the ground. He got no comfort, only a spanking for disobeying a parental injunction not to climb that tree. But now he was remembering something else. A few days later his father had picked him up in his arms, and . . .

Once beneath the shimmering canopy of the cherry tree, my father pulled a small branch towards me and let me hold it in my fist while we traded glances and smiles. Before he drove off to work, he used his pocketknife to cut a sprig, which he put in a tumbler of water, placing this on the little table where I ate my meals. . . . I knew I was both understood and forgiven. . . . This was my first mystical experience.

Camille had always had an abstract interest in genealogy, but late in his life he translated that into a new source of actual love. He discovered that his father had a whole network of French cousins, and through correspondence and travel he made relationships with them. Out of this relational archeology he created the warm extended family his painful childhood had denied him, mining memories of time past and investing them in real people in time present. This is one form of resilience. In his renewed religious involvement and his new relatives, he was finding fresh sources of strength, and fresh connections with his father. Creative writing and extensive psychotherapy promoted his discoveries of these lost loves; he is not alone in making good use of those activities to that end.

Despite such happy transformations as Camille’s, however, at the end of the day Conrad is more right than wrong. When we contrasted the lives of the men whose childhoods were bleakest—the Loveless—with the lives of those whose childhoods were sunniest—the Cherished—poor childhood was very significantly associated with poor later adjustment. Still, while being unloved as a child is painful, it isn’t the bottom line. The bottom line is what being unloved does to a child’s capacity to be loved, and to be loving, later. Some children manage to develop this capacity in spite of everything. But the ones who do not learn to love, or cannot let themselves be loved, do indeed live lives of woe.

SURPRISING WAYS THAT CHILDHOOD DID NOT AFFECT OLD AGE

Prospective studies of normal development are only now attaining the many decades they need to accompany adolescents into maturity, let alone old age. But in the process they are demolishing many cherished assumptions. We all “know,” for instance, that childhood affects the well-being of adults. But recent scientific reviews reveal that many popular explanations of how this happens—for example, through the loss of a parent—are rather less conclusive than we have long thought. 22

It’s not hard to explain the way people turn out—however they turn out—after the fact. I can evidence a crazy aunt, a rejecting mother, a clubfoot, a bad neighborhood, or any other circumstance I like to explain a poor outcome. And I can prove anything at all with single case studies. That is why I accompany the biographies here with their statistical context, and why I harp on how important it is to keep looking for statistical validation, even though we may not always like what it shows us.

Now, therefore, having used Study data to illustrate prospectively assessed childhood contingencies that were statistically important, I want to discuss some commonly held beliefs that are not supported by the data on the College (and sometimes the Inner City) men. It is very important for developmental psychologists to keep careful track of the interface between theory and fact, and to strive to bring the two into ever closer agreement. That is our best hope of keeping the children of the future from the devastating effects of destructive childhoods. It is true, and it is a hopeful truth, that some of the men with the worst childhoods enjoyed the period of end-of-life. Still, sixty years is a very long

time for a Sam Lovelace to have to wait for happiness—and even then his happiness was only relative. Furthermore, whereas a warm childhood, like a rich father, tends to inoculate a man against future pain, a bleak childhood is like poverty; it cannot cushion the difficulties of life. Yes, difficulties may sometimes lead to post-traumatic growth, and some men's lives did improve over time. But there is always a high cost in pain and lost opportunities, and for many men with bleak childhoods the outlook remained bleak until they died, sometimes young and sometimes by their own hands. We must get better at intervening when childhood conditions bode ill for the future, and that means finding ways to recognize them, not by sentiment but by data-based understandings of development and its antecedents.

The Grant Study, which approaches development from the front rather than from the back, has called many popular theories into question. The early Study staff did examine the effects of strict vs. liberal toilet training, and Earnest Hooton concluded in *Young Man, You Are Normal* that toilet training seemed to be entirely without significance for future behavior. 23 Hooton's book was published in 1945; sixty-five years of further study has done nothing to prove him wrong. Freud's theories about the deleterious effects of strict toilet training were based on retrospection, in the form of the memories of his usually middle-aged patients; yet again, prospective studies rule!

More recently, other childhood conditions that have been thought at times to have powerful implications for the future have failed to correlate significantly with the events of the Decathlon. For example, the pre-Spock Grant Study men were disciplined strictly by their parents. Eighty-six percent of them were breast-fed, but more than 50 percent were out of diapers by eighteen months. Thumb-sucking was inhibited with aluminum mitts and bitter aloes, even sometimes by tying an infant's arm to his body so as to keep hands away from mouth. None of this mattered. Physical health in childhood didn't hold up. Neither did the distance in age between the subject and the next child. Birth order was insignificant too, except that being the eldest child was correlated with occupational success. Even the death of a parent was relatively unimportant predictively by the time the men were fifty; by the time they were eighty, men who had lost parents when young were as mentally and physically healthy as men whose parents had lovingly watched them graduate from high school.

Even that old standby, the cold, rejecting mother, failed to predict late-life emotional illness or poor aging. It was good not to have mentally ill relatives, since genes so often trump environment. But basically it wasn't the absence of a loving mother that made all the difference, but the presence of one, or of an adored father, or of an otherwise warm childhood surround. This is what I mean when I say that our lives are shaped more by what goes right than by what goes wrong.

Another common assumption, based on retrospective "evidence," is that alcoholism is the result of an unhappy childhood. Certainly both alcoholics and clinicians may point blaming fingers in that direction after the fact. But prospective evidence suggests that this may be a reversal in which the results of alcoholism are being misconstrued as its cause. Alcoholism is passed on genetically, not environmentally, which means that an alcoholic parent appears in many alcoholics' backgrounds. 24 Alcoholism in a parent certainly ups the likelihood that childhood will be unhappy, but in that case the unhappy family is the cart; the horse constitutes the genes conducive to alcoholism. And as we shall see in Chapter 9, alcoholics' memories of their childhoods may be unwittingly altered either by physiologic changes due to alcoholism or defensively, to reduce guilt.

Another example. Ten years ago, I believed that the Loveless died sooner because they were careless of their own well-being. I wrote in *Aging Well*, "The Loveless, believing that they belonged to no one, failed to remember the old song's advice to 'button up your overcoat when the wind blows free.' " In this I was wrong; today's data reveal that the Loveless and the Cherished differ only slightly in the way they take care of themselves, at least with regard to the common vascular health risk factors of smoking, blood pressure, weight, and diabetes.

CONCLUSION

First, contrary to what might be called the developmental failure model of psychopathology, in the Harvard Study of Adult Development it was the men's successes, not their failures, that predicted subsequent mental health. What they did with a loving or bleak childhood had as much to do with future success as the childhood itself. Of the twenty-six personality traits that Woods assessed when the Grant Study men were in college, it was the one called Practical, Organized that best predicted objective mental health at ages thirty through fifty. The Terman Study also found that prudence, forethought, willpower, and perseverance in junior high school were the best predictors of vocational success at age fifty. 25 It's hard not to think that these are precisely the traits people need to find ways around failures, and make the

most of successes when they come along.

Second, it was not a disturbed relationship with one parent as much as a globally disturbed childhood, like those endured by Camille and Lovelace, that affected adult adjustment. Men from bleak childhoods were more likely than the others to be pessimistic and self-doubting; perhaps it was this that made many of them unable to take love in even when it was offered, or fearful about offering love themselves. These dynamics are illustrated to some degree by Sam Lovelace, and even more grimly by Peter Penn in Chapter 6 and Bill Loman in Chapter 8. The Gluecks' and other prospective longitudinal studies have shown that the children of multiproblem households are likely to be severely impaired in their later capacity to work. 26 For the upper-middle-class Grant Study men, however, inability to love seemed to be a more sensitive reflector of bad nurturing than work problems. Their childhoods were not so badly disrupted as those of the less fortunate Glueck men, and even relatively poor nurture did not in any way impair the capacity of many of these College men to excel in their jobs. (Of course, these particular men were all preselected as good workers; if they hadn't been, they wouldn't have been at Harvard.) But it did impair their capacity to love and be loved.

Men from poor childhoods were less able to deal consciously with strong emotions, either pleasurable or distressing ones; perhaps for this reason—that they couldn't grapple with strong feelings directly—they were more likely to turn to drugs to soothe themselves.

So, while an isolated trauma or a bad relationship need not in itself condition adult psychopathology, the Harvard Study of Adult Development makes clear that global disruptions of childhood have strong predictive power, none of it good. Children who fail to learn basic love and trust at home are handicapped later in mastering the assertiveness, initiative, and autonomy that are the foundation of successful adulthood. Prevention will be best served when seriously troubled families can be accurately identified and made the foci of special concern. Mental illnesses and alcoholism create conditions that can ravage entire families, and can destroy children's futures for decades to come.